

PRN's for agitation/psychosis

PO (mild to moderate)	IM (moderate to severe)	IV	
Haldol 5mg Q6 (60min to effect) can admin every 30mng	Haldol 5mg +ativan 2mg (15min effect) calm	yes	#The Go-To if limited info #Max near 40mg Qd #2, 5 for elderly
B52	Haldol 5-10, Ativan 2, Benadryl 50mg		#Severe agitation
Olanzapine/Zydis 5mg Q6	Olanzapine 5mg Q6		#Max around 30QD #avoid in high BMI and old age
Ativan 2mg Q6	Ativan 2mg Q6		#Used alone in PCP and Meth intoxication #Watch for paradoxical rxn, delirium
Abilify 2 Q6	Zyprexa 5mg or Geodon 20mg		Not common
Geodon 10-20mg Q6	Geodon 10-20mg Q6		#Good for hight BMI #High QT prolongation
G52	Geodon 20, Ativan 2, Benadryl 50		
Thorazine 50mg Q6	Thorazine 50mg Q6		#800mg max #Can start at 25 if worried #watch for orthostatics
Risperdal	Haldol or prolixin 5mg Q6		
Prolixin 5mg Q6	Prolixin 5mg or Haldol		#if planning on deconoate (IM Q2wks) #Good if have "haldol allergy" and don't like zyprexa
Clozapine	Olanzapine 5mg Q6		

++EPS:**Dystonia**: minutes to 2 days, neck, tongue, mouth, eye deviation, tx is benadryl 50, benadryl 50, lorazepam 2mg, (due to D2 blockade, tx is Ach blockade)

Parkinsonism: approx 1wk, add benztropine

Akathisia: approx 1wk

Tardive Dyskinesia: years

NMS: Anytime, AMS, Rigidity, hyperthermia, tremor, autonomic instability, tachy, diaphoresis, incontinence, sialorrhea, mydriasis, Increased CK, WBC, CR,

BUN

Tx: IVF, ice packs, urine alkalinization, Dantrolene or bromocriptine (2.5mg TID, increase to 5-10TID)